

Product Registration Number:

MAL19970290AZ

Further information can be obtained from your doctor or pharmacist.

ROYCE®

Product Holder / Manufactured by:

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ROYCE®

DICODINE TABLET
(Dihydrocodeine Bitartrate - Analgesic)

Presentation:

A white coloured, round, shallow biconvex tablet, a breaking line on one side of the tablet and APP marking on the reverse side.

Content:

Each tablet contains Dihydrocodeine Bitartrate 30 mg.

Pharmacological Information:

Dicodine Tartrate like other narcotic analgesics has the similar mechanism of action. They have the analgesic properties by binding with stereospecific receptors at any sites within central nervous system (CNS) to alter both the perception of pain and the emotional response to pain. Precise sites and mechanisms of action have not been fully determined.

It has been proposed that there are multiple subtypes of opioid receptors, each mediating various therapeutic and / side effects of opioid drugs. The actions of an opioid analgesic may therefore depend upon its binding affinity for each type of receptor and whether it acts as a full agonist or is inactive at each type of receptor. At least two of these types of receptors (μ and κ) mediate analgesia. A third type of receptor (σ) may not mediate analgesic, actions at this receptor may produce the subjective and psychotomimetic effects characteristic of opioids having mixed agonist / antagonist activity.

Dihydrocodeine Tartrate is well absorbed after oral administration. It crosses the placenta.

After an oral dose of 52mg, 20 to 30% is excreted in the urine in 24 hours after an intramuscular dose of about 43mg. 40 to 60% is excreted in the urine over the same period; the excretion of the oral dose is increased to about 35% when the urine is acid; 30 to 45% of the urinary excreted material is conjugated.

Indication:

Dicodine Tablet is used to relief moderate to severe pain. The action of dihydrocodeine tartrate is almost purely analgesic-it is virtually free from sedative or hypnotic effects. In a study of oral analgesics for the relief of chronic pain, 30mg dose was found to be equivalent to 100mg of pethidine. In the trial, Dihydrocodeine Bitartrate has a lower incidence of side effects than either morphine or pethidine.

It is fully active when administered orally and in the recommended doses, causes little or no respiratory depression, in addition to being a potent analgesic the compound also exhibits well-defined antitussive activity.

Dicodine Tablet is indicated in all painful conditions where an alert patient is desired, viz. sciatica, osteoarthritis, chronic rheumatoid arthritis, arthritis of the spine, peripheral vascular disease, post-herpetic neuralgia, Pagets disease, malignant disease, post-operative pain.

Because Dicodine Tablet in the recommended doses, causes little or no respiratory depression, its use in the treatment of post-operative pain may reduce the risk of chest complications.

Dosage and Administration:

Dicodine Tablet is best administered with or after food. It is not recommended for administration to children under 4 years. Adults: 1 tablet (30 mg) every four to six hours or as directed by the physician. Children over 4 years: 1 mg per kg body weight every 4-6 hours.

Contraindication:

Respiratory depression; obstructive airways disease.

Warning and Precautions:

As dihydrocodeine may bring about histamine-release, Dicodine Tablet should not be given during an attack of asthma and it should be administered with due care to persons liable to such attacks.

Dosage should be reduced in the elderly, in hypothyroidism and in chronic hepatic disease. Alcohol should be avoided whilst under treatment with Dicodine Tablet.

Notice should be taken of current views that any drug should be prescribed with caution during pregnancy. Prolonged use of high doses of dihydrocodeine may produce dependence of the morphine type. Liability to abuse is about the same as for Codeine.

Interactions with Other Medicaments:

Other narcotic analgesics; sedative-hypnotics, anti-anxiety agents, general anesthetics, and other CNS depressants (including alcohol); MAO inhibitors; tricyclic anti-depressants - Increases CNS depression; reduce dose of one or both agents if used concomitantly or in close succession. Anti-cholinergic agents - increase risk of paralytic ileus.

Pregnancy and Lactation:

Dihydrocodeine has not been reported to be excreted in breast milk; however it is advisable to caution the mothers.

Side Effects:

Constipation, nausea, vomiting, headache and vertigo occur and are relatively more common when the dose is increased above 30 mg. If constipation occurs it can be treated with a gentle laxative.

Symptoms and Treatment of Overdose:

Symptoms of overdose will be manifested as more severe in side-effects. Conservative management for overdosage is recommended, gastric lavage should be carried out. Severe respiratory depression can be treated with naloxone hydrochloride 0.4mg subcutaneously, repeated as required at 2 or 3 minutes intervals.

Storage Condition:

Keep container tightly closed. Store in dry place (below 30°C). Protect from light.

Pack Size:

Blister pack of 1 x 10's, 10 x 10's, 50 x 10's and 100 x 10's.